

1. Administrative & Study Identification

Study Title: A Study to Evaluate the Efficacy and Safety of Obinutuzumab in Participants With Systemic Lupus Erythematosus
Official / Full Study Title: A Phase III, Randomized, Double-Blind, Placebo-Controlled, Multicenter Study to Evaluate the Efficacy and Safety of Obinutuzumab in Patients With Systemic Lupus Erythematosus
Short Title/Acronym: ALLEGORY Protocol
Number: CA42750 / 2023-504774-38-00 **NCT Number:** NCT04963296
Posting ID: 132655156338700294 **Sponsor / Company Name:** Roche (F. Hoffmann-La Roche Ltd / Genentech)
Investigational Product: Obinutuzumab (Gazyva / Gazyvaro)
Phase of Development: Phase III (Phase Requirement: Trial must be in PHASE3)
Medical Monitor: Clinical Operations Lead (Hoffmann-La Roche)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of obinutuzumab versus placebo, measured by the percentage of participants achieving a Systemic Lupus Erythematosus (SLE) Responder Index four (SRI) response at Week 52. **Secondary Objectives:** To evaluate the achievement of SRI(6), sustained corticosteroid control, and time to first BILAG flare.

2.2 Background & Rationale Systemic lupus erythematosus (SLE) is a chronic, relapsing, inflammatory, and often febrile multisystemic disorder of connective tissue, characterized principally by involvement of the skin, joints, kidneys, and serosal membranes. It is of unknown etiology, but is thought to represent a failure of the regulatory mechanisms of the autoimmune system. The disease is marked by a wide range of system dysfunctions, an elevated erythrocyte sedimentation rate, and the formation of LE cells in the blood or bone marrow. Current standard-of-care therapies (e.g., corticosteroids, conventional immunosuppressants) often have incomplete efficacy and limiting toxicities. Obinutuzumab directly targets B-cell depletion to address the high unmet medical need in patients with active, autoantibody-positive SLE by evaluating whether it can effectively control disease activity and delay organ damage.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1

3.2 Planned Enrollment & Duration Trial Status:

ACTIVE_NOT_RECRUITING (Active, not currently recruiting) **Targeted Enrollment (\$N\$):** 303 participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 14-Jul-2021 **Estimated Primary Completion:** 06-Mar-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosis of SLE according to the 2019 EULAR/ACR Classification Criteria ≥ 12 weeks prior to screening. 2. Active disease on Day 1 defined as a SLEDAI-2K score ≥ 8 and Physician's Global Assessment (PGA) score ≥ 1.0 on a 0 to 3 visual analogue scale (VAS). 3. Autoantibody positive (ANA $\geq 1:80$, or anti-dsDNA and/or anti-Sm antibodies above the ULN, as determined by the central laboratory). 4. Low C3 or low C4 or low CH50 complement as determined by the central laboratory. 5. High disease activity at screening based on BILAG-2004 (Category A in ≥ 1 organ system and/or Category B in ≥ 2 organ systems), SLEDAI-2K ≥ 8 , and PGA ≥ 1.0 . 6. Current receipt of ≥ 1 standard therapies (oral corticosteroids [OCS], antimalarials, conventional immunosuppressants) at stable doses.

4.2 Exclusion Criteria 1. Pregnancy or breastfeeding. 2. Presence of significant lupus-associated renal disease and/or renal impairment. 3. Receipt of excluded therapies, including any anti-CD20 or anti-CD19 therapy less than 9 months prior to screening; or cyclophosphamide, tacrolimus, ciclosporin, or voclosporin during the 2 months prior to screening. 4. Known active infection of any kind or recent major episode of infection. 5. Intolerance or contraindication to study therapies.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Obinutuzumab 1000 mg or matched placebo administered on Day 1 and at Weeks 2, 24, and 26. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** 52 weeks (double-blind period), followed by an open-label extension period for up to 104 weeks.

5.2 Concomitant, Prohibited & Associated Therapies Permitted: Stable standard-of-care therapy (antimalarials, conventional immunosuppressants, and oral corticosteroids). **Associated/Mapped Background Therapies:** Symptom management drugs such as Paracetamol (ATC Code: N02BE01). **Prohibited:** Anti-CD20/anti-CD19 therapies, cyclophosphamide, tacrolimus, ciclosporin, or voclosporin during the specified screening and study windows.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, SLEDAI-2K/BILAG scoring, Autoantibody & Complement Labs
Baseline	Day 1	Randomization, First IV Infusion (Obinutuzumab or Placebo)
Interim	Weeks 2, 24, 26	Subsequent IV Infusions; Disease activity and safety assessments conducted every 4 weeks
End of Study	Week 52	Primary Endpoint Assessment (SRI-4), Safety Labs, Transition to Open-Label Extension

7. Outcome Measures (Endpoints)

7.1 Primary Outcome 1. Percentage of Participants who Achieve Systemic Lupus Erythematosus Responder Index (SRI) at Week 52:

Requires reduction from baseline of ≥ 4 points in the SLEDAI-2K, no new systems or organs affected (defined by ≥ 1 new BILAG A or ≥ 2 new BILAG B items compared with baseline using BILAG-2004), and no worsening from baseline of ≥ 0.30 points on a 3-point Physician's Global Assessment Visual Analogue Scale (PGA-VAS).

7.2 Secondary Outcomes 1. Percentage of Participants who Achieve SRI(6) at Week 52. 2. Percentage of Participants Entering the Study on Prednisone ≥ 10 mg/day (or equivalent) who Achieve Sustained Corticosteroid Control. 3. Time to First BILAG Flare over 52 Weeks.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection during site visits every 4 weeks, with special monitoring for infusion-related reactions and infection-related adverse events. **Serious Adverse Events (SAEs):** Evaluated continuously; standard 24-hour reporting timelines apply for life-threatening events or hospitalizations. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee utilized to continually oversee patient safety and trial integrity.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy endpoints; Safety Set for all AE/SAE evaluations. **Sample Size Justification:** $N = 303$ participants, powered appropriately ($\alpha = 0.05$, $\text{Power} = 80\%$) to detect a statistically significant difference in SRI-4 response rates between the Obinutuzumab and Placebo groups. **Handling of Missing Data:** Standard imputation methods (e.g., Non-Responder Imputation).

[NRI] for binary efficacy endpoints like SRI-4) applied as per protocol.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring to ensure protocol adherence, especially regarding consistency in clinical scoring (BILAG/SLEDAI-2K). **Audit Trail:** eCRF locking and data clarification forms managed securely per the sponsor's clinical data management protocols.

11. Study Identifiers & Governance

Primary ID: CA42750 **Registry Number:** NCT04963296 **Posting ID:** 132655156338700294 **Sponsor/Lead Organization:** Roche **Study Title:** A Study to Evaluate the Efficacy and Safety of Obinutuzumab in Participants With Systemic Lupus Erythematosus **Official Regulatory Title:** A Phase III, Randomized, Double-Blind, Placebo-Controlled, Multicenter Study To Evaluate The Efficacy And Safety of Obinutuzumab in Patients With Systemic Lupus Erythematosus

12. Clinical Framework (Systemic Lupus Erythematosus Specific)

Disease Area / Condition: Systemic Lupus Erythematosus (SLE)
Common Name: Lupus **Preferred Name (UMLS):** Lupus Erythematosus, Systemic **Semantic Type:** Disease or Syndrome **Classification Codes:** MeSH: D008180 | SNOMED ID: 55464009 | HPO Code: HP:0002725
Diagnosis Threshold: 2019 EULAR/ACR Classification Criteria, SLEDAI-2K ≥ 8 , PGA ≥ 1.0 , and positive autoantibodies
Medical Methodology: Targeted B-cell depletion using a Type II anti-CD20 monoclonal antibody (Obinutuzumab) combined with Standard of Care therapies.

13. Study Procedures & Methodology

Management Pathway: Addition of obinutuzumab or placebo infusions to stable standard-of-care maintenance therapy. **Education Protocol (HCP):** Standardized training for investigators on SLEDAI-2K and BILAG-2004 clinical scoring criteria. **Education Protocol (Patient):** Education on recognizing signs of infusion-related reactions and active infection. **Quality Audit Mechanism:** Centralized laboratory review for autoantibodies/complement levels and independent adjudication of clinical disease activity scores.

14. Observation & Follow-Up Schedule

Total Duration: 52 weeks (double-blind) + up to 104 weeks (open-label) **Onsite Visit Cadence:** Day 1, Weeks 2, 24, and 26 for dosing; clinical assessments every 4 weeks through Week 52 **Remote Monitoring Frequency:** Between scheduled clinical assessment visits as required. **Checklist Review Interval:** Every 4 weeks (Q4W) for disease activity and safety monitoring.

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[07-Apr-2026]					
Clinical Operations Lead	[Name]	_____	[07-Apr-2026]					Lead
Statistician	[Name]	_____	[07-Apr-2026]					